

Common Seizure Mimics

Everything that shakes isn't a seizure. Before you determine the patient has experienced epileptic seizures, make sure to have considered relevant seizure mimics. These nonepileptic conditions imitate epileptic seizures and recurrences may be misdiagnosed as epilepsy.

A careful history, physical examination, and video EEG evaluation are useful in diagnosing the cause of paroxysmal "seizure-like" events. Less commonly, both epileptic and nonepileptic events may coexist, necessitating inquiry into whether the event in question is consistent with the patient's typical event and if other uncharacterized events also exist. A complete evaluation should aim to capture and characterize each of the patient's event type, though frequent events are easier to characterize than those occurring seldomly.

In a majority of conditions that imitate epileptic seizures, the EEG remains unchanged during the event. However, some nonepileptic conditions are associated with abnormal EEG patterns that should not be mistaken for ictal activity. This chapter lists common seizure mimics and their associated electrographic features.

Seizure mimics are grouped as physiological (organic disease) or psychological. Furthermore, physiological mimics may be neurologic, systemic, or both (such as syncope) [1].

Seizure Mimics

Neurological Mimics

- syncope
- transient ischemic attack (TIA)
- migraines
- parasomnias
- movement disorders